

Christopher Zaragoza Mabini, DO, MSAEd

MIGS Fellow, Department of Gynecology

PRIME Healthcare — St. Francis Hospital

355 Ridge Avenue, Evanston, IL 60202

March 24, 2026

cc:

Dr. Sarasam, Saint Francis Hospital-Evanston

Mr. Adamo, Hospital Administration, Saint Francis Hospital-Evanston

Dr. Maria Teresa Tam, MD, Program Director, MIGS Fellowship

Risk Management, Prime Healthcare Management, Inc.

Re: Request for Written Confirmation of Professional Liability Coverage Scope and Clarification of On-Call Coverage Structure — Resident Employment Agreement, PGY-7 MIGS Fellowship

Dear Risk Management Department:

I. PURPOSE

I write to formally request written clarification and confirmation regarding the scope of professional liability insurance coverage applicable to me under my Resident Employment Agreement (the "Agreement"), effective August 14, 2025, between Saint Francis Hospital-Evanston, LLC ("Hospital") and myself, Christopher-Armand Zaragoza Mabini, DO, appointed as a PGY-7 Minimally Invasive Gynecologic Surgery (MIGS) Fellow.

This request is prompted by two circumstances: (1) a patient safety near-miss event on March 16, 2026, the details of which have been submitted separately to Dr. Sarasam and Mr. Adamo; and (2) a meeting held on March 24, 2026, during which significant questions were raised

regarding the structure, scope, and administrative ownership of Gynecology on-call coverage at this institution. Together, these circumstances have created material ambiguities regarding the scope of my coverage under the Agreement that I respectfully ask to have resolved in writing before continuing to render clinical services.

II. CONTRACTUAL BASIS

Section 1.8 of the Agreement provides professional liability insurance coverage of no less than \$1,000,000 per claim and \$3,000,000 in the aggregate, as well as tail coverage for claims reported or filed following completion of the training program. Coverage applies to acts performed "within the scope of [the Resident's] duties and responsibilities of the training program." Section 1.8 further provides that moonlighting services — whether performed at the Hospital for a third-party entity or at an outside facility — are not covered under the Hospital's professional liability or general liability policies. Section 5.2 requires that any approved moonlighting activities be supported by separate and adequate professional liability insurance, with the Resident agreeing to indemnify and hold the Hospital harmless from any liability arising therefrom.

Sections 1.1, 1.5(a), and 1.5(d) establish that my schedule of specific assignments and call schedule are subject to modification by the Hospital "in consultation with the Program Director," and that clinical and educational work hours are determined by the Program Director. These provisions define how on-call assignments are established and, therefore, what acts fall "within the scope" of the training program for purposes of §1.8 coverage.

III. FACTUAL BACKGROUND

On March 16, 2026, at approximately 23:46, I received a PerfectServe notification regarding an urgent Gynecology consultation in the Emergency Department for a patient with a ruptured ectopic pregnancy requiring emergent surgical intervention. At that time, I was not listed as the designated on-call Gynecology provider in Lightning Bolt, and I had not received a call schedule for March 2026. I was not on call under my fellowship schedule.

During the March 24, 2026 meeting, Hospital administration represented that Dr. Oldham serves as the on-call Gynecology attending during my fellowship call periods, and stated that the ED had "coverage" for March. However, the institutional scheduling system — Lightning Bolt — contained no designated on-call Gynecology provider for March 16, 2026, for either Dr. Oldham

or myself. No call schedule for March 2026 was communicated to me or entered into Lightning Bolt by any party. When I raised the question of why my name appeared as the backup on-call GYN provider on a date when Dr. Oldham was represented to be the on-call attending, no explanation was provided. When I asked who the designated on-call provider is for the remainder of March 2026, that question also went unanswered.

I wish to briefly address the representation made during the meeting that the scheduling gap resulted from a failure by the fellow and Program Director to "check the schedule" and "coordinate." I note respectfully that one cannot check a schedule that was never created. The responsibility for maintaining the Gynecology on-call schedule in Lightning Bolt belongs to the Department Chair and the institution. When Dr. Sarasam suggested during the meeting that I had failed to communicate my availability to Dr. Oldham, I noted that I had emailed my availability to Dr. Oldham on December 21, 2025, through the end of March 2026. As that email was not requested or reviewed during the meeting, I have since forwarded the correspondence to Dr. Sarasam as documentary support. This evidence was available had it been requested at the time.

I also note that the following questions raised during the meeting were not addressed and remain outstanding: (a) why my name was listed as the default backup GYN provider on March 16 if Dr. Oldham was the designated on-call attending; (b) who the designated on-call GYN provider is for the remainder of March 2026; (c) the identity of the individual who will be responsible for scheduling in the new Amnion system; and (d) how "coverage" can be said to have existed for March when no provider was entered into Lightning Bolt. These unresolved questions bear directly on the coverage and liability issues that are the subject of this correspondence.

During the meeting, Hospital administration stated that as of April 1, 2026, a designated individual would be assigned responsibility for entering Gynecology schedules into the Amnion scheduling system. However, the identity of that individual could not be provided when directly asked. I respectfully note that the absence of this information raises a practical concern about whether a durable administrative solution is in place or whether the scheduling gap that created the March 16 near-miss may recur.

Notwithstanding the above, I exercised my independent clinical judgment to present to the Emergency Department, evaluate the patient, and proceed to the operating room, where I performed the emergent surgical procedure without complication. The patient received timely care as a result of this decision.

The foregoing circumstances raise a material question: because no verified on-call assignment placed me on call under my fellowship schedule on March 16, 2026, were my clinical actions performed "within the scope of [my] duties and responsibilities of the training program" as required by §1.8 for coverage to attach?

IV. REGULATORY CONTEXT

I note for the record the regulatory framework within which these questions arise.

The Emergency Medical Treatment and Labor Act (EMTALA), 42 U.S.C. §1395dd, requires Medicare-participating hospitals to provide appropriate medical screening examinations and stabilizing treatment for individuals presenting with emergency medical conditions. Under 42 C.F.R. §489.20(r)(2), hospitals must maintain a current on-call list identifying individual physicians by name. CMS Interpretive Guidelines (State Operations Manual, Appendix V) specify that this list must be accurate, up-to-date, and reflect current physician privileges. An unfilled scheduling system does not satisfy this requirement.

Under 42 C.F.R. §489.24(j), hospitals must have written policies and procedures addressing on-call physician availability. The CMS Interpretive Guidelines provide that when on-call coverage for a specialty is unavailable, transfer may be appropriate because the medical benefits outweigh the risks. Civil monetary penalties under 42 U.S.C. §1395dd(d) and 42 C.F.R. §1003.500 may reach \$119,942 per violation for both hospitals and individual physicians. Exclusion from Medicare and Medicaid participation and private civil liability under applicable state law are also potential consequences.

I raise this framework not to make any allegation, but to underscore the seriousness with which accurate on-call documentation and clear provider liability coverage must be maintained. The

absence of any designated Gynecology provider in Lightning Bolt on a night when a patient with a ruptured ectopic pregnancy presented to the ED is a matter warranting careful institutional attention. I further note that the obligation to maintain an on-call list under 42 C.F.R. §489.20(r)(2) is an institutional obligation of the Hospital. It is not delegable to a trainee and cannot be satisfied by informal assumptions about provider availability.

V. SPECIFIC CLARIFICATIONS REQUESTED

I respectfully request written confirmation from the Hospital and its professional liability insurer addressing each of the following:

1. March 16, 2026 Incident. Please confirm that all clinical actions I performed on March 16–17, 2026 — including patient evaluation in the Emergency Department and the emergent surgical procedure — are fully covered under §1.8 of the Agreement, including defense costs, indemnification, settlements, and judgments, notwithstanding the absence of a verified on-call assignment in Lightning Bolt at the time services were rendered.
2. Prospective Voluntary Emergency Responses. In the event that I respond to clinical emergencies at SFH in the future while not holding a verified on-call assignment in Lightning Bolt, please confirm whether such actions will be covered under §1.8, or whether I would be considered to be acting outside the scope of the training program and therefore without institutional malpractice coverage.
3. Additional Call Beyond Fellowship Assignment. My call obligation consists of three unpaid Gynecology ED call shifts per month at SFH, as determined by the Program Director and Designated Institutional Official (DIO) at the inception of the fellowship and reaffirmed by Prime Healthcare for the PGY-7 year. I am fulfilling this obligation in full. If the Hospital expects or requires additional GYN ED call shifts beyond these three — whether by departmental direction, hospital administration, or by virtue of my designation as a "default" GYN provider in the scheduling system — I request advance written clarification of the following before any such expansion:

(a) whether those additional shifts are classified as within the scope of the training program (covered under §1.8) or as moonlighting/outside professional activities (excluded from coverage under §1.8 and §5.2);

(b) what compensation, if any, would be provided, given that my annual stipend of \$86,398 (§2.1, Attachment B) was established in consideration of the fellowship duties defined by the Program Director — not in contemplation of supplemental institutional coverage responsibilities; and

(c) whether any such expansion requires a formal written amendment to the Agreement under §6.6.

I am willing to discuss any proposed expansion of my clinical responsibilities through the appropriate channels — with the Program Director and DIO per §§1.5(e) and 5.1 — but such expansion must include appropriate coverage classification, compensation, and must not compromise the educational objectives of the surgical fellowship.

4. Default Provider Designation. I have been informed that I am currently listed as the "default backup GYN provider" in the ED system for all dates on which no other GYN provider is scheduled. This designation was not established by the Program Director, is not reflected in the Agreement, and was not addressed when I raised it during the March 24 meeting. Please confirm whether clinical actions taken pursuant to this default designation are covered under §1.8. If they are not, I request immediate removal of this designation from all Hospital systems.

5. Tail Coverage. Please confirm that the tail coverage referenced in §1.8 extends to all clinical activities performed during my fellowship term that fall within the scope of the training program, including any claims arising from the March 16, 2026 incident that are reported or filed after my completion of the program.

6. Dr. Oldham's On-Call Status and Coverage Role. Hospital administration represented during the March 24 meeting that Dr. Oldham serves as the on-call attending during my fellowship call periods. Please confirm: (a) whether Dr. Oldham held a verified on-call assignment in Lightning Bolt for March 16, 2026; (b) the institutional understanding of how attending backup coverage interfaces with fellowship on-call assignments for purposes of §1.8 coverage; and (c) if Dr. Oldham was the designated on-call attending, why my name — rather than hers — was documented in the patient's record as the on-call provider and why I was contacted via PerfectServe.

7. Current March 2026 Coverage and Amnion System Transition. Please identify by name the designated on-call Gynecology provider(s) for each remaining date in March 2026. This information is operationally necessary: if I receive PerfectServe notifications or am contacted by the ED on dates when I am not on call, I need to know the correct on-call provider so that I may redirect staff appropriately. The March 8 and March 16 incidents both involved my name being documented as the responsible provider in the absence of a verified assignment — a circumstance preventable only by ensuring a clearly identified, named provider is entered in the scheduling system and communicated to all relevant departments.

Additionally, please confirm: (a) the name and title of the individual designated to enter and maintain the GYN on-call schedule in the Amnion system; (b) the date on which that responsibility formally takes effect; and (c) what interim measures are in place to ensure verified, uninterrupted GYN on-call coverage until the new system is operational.

VI. SCOPE OF DUTY — CLARIFICATION

During the March 24 meeting, Dr. Sarasam referenced language in the Agreement obligating me to perform duties "as the hospital needs," and appeared to invoke this language as a basis for expanding my clinical responsibilities to include additional daytime ED call coverage. I wish to respectfully address this interpretation.

First, the Agreement provisions governing my schedule (§§1.1, 1.5(a), 1.5(d)) consistently vest scheduling authority in the Program Director, not in Hospital executive leadership. That authority is referenced no fewer than three times in Section 1.5 alone. If the Agreement intended to grant the CEO or CMO the authority to assign clinical duties directly to fellows, it would do so expressly.

Second, §1.5(c) — which addresses continuity of patient care "not necessarily limited to scheduled hours" — is a continuity-of-care provision for patients already under my care. It is not a blanket availability clause for new ED consultations. Were the Hospital's interpretation correct, the on-call schedule would serve no purpose and there would be no need to maintain

one — a result that is irreconcilable with §1.1 (which specifically contemplates a call schedule) and with the Hospital's obligation under 42 C.F.R. §489.20(r)(2) to maintain a named on-call list.

Third, my appointment is as a PGY-7 MIGS Fellow in a surgical subspecialty program (§1.2). The educational purpose of this fellowship is to develop advanced competence in minimally invasive gynecologic surgery. Redirecting a surgical fellow to serve as the primary daytime ED Gynecology coverage provider would substitute institutional staffing needs for educational programming — an outcome that may conflict with the Hospital's obligation under §1.6 to provide "an appropriate learning and working environment" and with ACGME requirements protecting trainees from excessive non-educational clinical service demands.

Fourth, there is an internal inconsistency in the positions advanced during the meeting: the Hospital simultaneously claimed that Dr. Oldham is the designated on-call attending and that "coverage" exists, while asserting that I have a contractual obligation to respond to all GYN consultations regardless of on-call status. If Dr. Oldham is the on-call attending and coverage exists, no additional obligation on my part is necessary. If coverage does not exist and the Hospital requires me to fill that gap, the appropriate path is to acknowledge the gap as an institutional scheduling matter and address it through proper administrative channels — not to reinterpret the fellow's contract to create obligations the Agreement does not contain.

Fifth, I wish to briefly note the compensation context underlying this matter. Prior to Prime Healthcare's assumption of operations, Gynecology attending physicians received \$1,200 per ED call shift. That compensation was subsequently reduced to \$200 per shift — a reduction of approximately 83%. I raise this not as a grievance, but as context: the fellowship stipend of \$86,398 (§2.1, Attachment B) was established in consideration of the three call shifts per month determined by the Program Director and DIO — not in contemplation of supplemental coverage responsibilities arising from a reduction in attending call compensation.

Regarding the suggestion that Prime Healthcare's funding of the fellowship grants Hospital administration authority to expand my clinical duties unilaterally: the Agreement is a bilateral contract that defines specific duties (§1.5), vests scheduling authority in the Program Director (§§1.1, 1.5(a), 1.5(d)), establishes compensation in consideration of those duties (§2.1, Attachment B), requires modification only by mutual written agreement (§6.6), and constitutes the complete agreement between the Parties (§6.7). Funding source does not convert a fellow

into a general employee whose duties may be expanded at the discretion of hospital administration without regard to the Agreement's terms, the Program Director's authority, ACGME duty hour protections, or the educational purpose of the fellowship.

Dr. Sarasam also referenced "Prime Bylaws" as authority for mandating additional coverage. Section 1.7 does bind the Resident to Hospital rules, regulations, and policies — but only "except as specifically modified herein." The Agreement specifically modifies the general employee framework by vesting scheduling authority in the Program Director (§§1.1, 1.5(a), 1.5(d)), by requiring PD and DIO approval for outside professional activities (§§1.5(e), 5.1), and by capping total duty hours at eighty per week (§5.1). These specific contractual terms take precedence over general bylaws under the express language of §1.7. Moreover, §6.7 provides that the Agreement supersedes all prior agreements and understandings. General bylaws that were not incorporated by specific reference and that contradict the Agreement's express terms do not override its provisions.

If the Hospital requires additional GYN ED coverage beyond the three call shifts established by the Program Director and DIO, the appropriate mechanism is a formal written amendment under §6.6 — with mutual consent, corresponding compensation, and appropriate coverage classification — or a separate, independently insured compensated arrangement. Any such expansion would also require advance written approval of the Program Director and DIO per §§1.5(e) and 5.1, consistent with ACGME Common Program Requirements, Section VI.

VII. BASIS FOR THIS REQUEST

I raise these questions in good faith, and in the same spirit of commitment to patient care that I demonstrated on March 16, 2026, when I voluntarily presented to this Hospital — while not on call — to provide emergent surgical care to a patient who required immediate operative intervention. The events of March 16 and the discussions of March 24 have made clear that material ambiguities exist regarding on-call coverage structure, the delineation of my clinical responsibilities, and the scope of professional liability coverage for services rendered outside of a verified fellowship on-call assignment. Resolving these ambiguities in writing will protect both the institution and its providers and ensure that the systems supporting patient safety are clearly defined.

I am fully committed to this fellowship and to providing excellent care at Saint Francis Hospital, and I look forward to a constructive resolution of these matters.

I respectfully request written responses to the above inquiries no later than fourteen (14) business days from the date of this letter.

VIII. PRESERVATION OF RIGHTS

This request is made without waiver of any rights or remedies available to me under the Agreement, applicable federal or state law, or any institutional policies — including, without limitation, the grievance and due process procedures set forth in Section 4 of the Agreement, the dispute resolution provisions of §4.2 and Attachment C, and all rights preserved under Attachment C, Section 5. All such rights are expressly reserved.

I am available to discuss these matters at your earliest convenience and may be reached via PerfectServe or email.

Respectfully,


Christopher Zaragoza Mabini, DO, MSAEd

MIGS Fellow, Department of Gynecology

PRIME Healthcare — St. Francis Hospital

Evanston, Illinois

References:

1. Resident Employment Agreement, PGY-7, effective August 14, 2025
2. Correspondence to Dr. Sarasam and Mr. Adamo re: March 16, 2026 Incident Report
3. Email correspondence to Dr. Oldham dated December 21, 2025, documenting availability submission through March 2026 (forwarded to Dr. Sarasam on March 24, 2026)